

20th January 2023

Prof. Judith Kimiywe
Director Center For Research Ethics and Safety
Kenyatta University Ethics Review Committee (KUERC)
P.O. Box 43844 – 00100
Nairobi, Kenya

Dear Prof. Kimiywe:

**Re: Amendments to Conditionally Approved Protocol Number PKU/2627/E1752:
Testing Pathways to Scale and Poverty Alleviation Effects of a Cost Effective and
Evidence-Based Mental Health Innovation (Anansi)**

I want to thank you and KUERC for your careful and thoughtful review of our application (ref. no: PKU/2627/E1752) for our study entitled “Testing Pathways to Scale and Poverty Alleviation Effects of a Cost Effective and Evidence-Based Mental Health Innovation (Anansi)”. In addition, we want to thank you for granting us conditional approval as per your letter dated 16th November 2022.

Below, we respond to you and the committee’s comments and wonderful guidance on our project. Should we have missed anything, we would be very happy to make further revisions. We look forward to your continued guidance:

1. Study design is not clear as nowhere in the design does the researcher say it is an experimental study. Is it an experiment, RCT, or what?

We thank you for the opportunity to paint a clearer picture of our study. This is an experimental trial comprising of three studies that build on each other. Fundamentally, the key question that we are answering is: ***Does it matter who delivers the Shamiri intervention?*** In other words, is there a difference on intervention outcomes if it is delivered by the organization that developed it (i.e., Shamiri intervention) or an implementing partner (sometimes called delivery through a “train-the-trainers” model). We answer this question in:

- **Study 1:** Here, the Shamiri intervention is delivered by the Shamiri Institute team (we call this a “centralized” dissemination model). Students are randomized to receive the Shamiri intervention or to an enhanced treatment as usual control group.
- **Study 2:** Here, the Shamiri intervention is delivered by the Africa Mental Health Research and Training Foundation (AMHRTF) who are trained through the “train-the-trainers” model (we call this a “decentralized” dissemination model). Students are randomized to receive the Shamiri intervention or to an enhanced treatment as usual control group.
- **Study 3:** Here, we directly test the centralized vs decentralized models through a design where youths, in the same schools, are randomized to receive Shamiri through Shamiri Institute (centralized model) or AMHRTF (decentralized model).

We hope that this clarification paints a clearer picture of our experimental study. We have added this to the proposal on page 10, highlighted in yellow for reference.

2. The justification provided for studying these schools is inadequate – they are understudied.

We aim to enroll secondary schools in Nairobi, Kiambu, Makueni, and Machakos for several reasons. In addition to the fact that these areas require more research attention and will allow us to focus our work on low-resource settings, we are testing whether our model and intervention works to improve the mental health of adolescents. Most adolescents are enrolled in secondary schools which makes implementation and data collection in such settings convenient. Second, we and our implementing partner already have established infrastructure to implement in these areas. Third, our existing relationship with these schools eases our entry and delivery of our program. This clarification has been added to the proposal on page 9.

3. How will the learners/youth be classified as low income, yet there will be no exclusion criteria in the study?

For clarity, our study will include a universal sample and no exclusion criteria will be applied. As such, there will be no classification of youths based on socio-economic status. Where we refer to low-income, we only refer to the macro-level observation, which is that Kenya is classified by the World Bank as a low – and middle – income country. In fact, we aim to a wide variety of diverse schools including secondary schools as per the Ministry of Education’s classification ensuring we have schools that range from sub-county schools to extra-county and national schools.. This diverse sample will strengthen the generalizability of our study to the larger and diverse broader population of adolescents in Kenya. This is described in detail on page 9 of the proposal.

4. The study will utilize group leaders from high school and colleges yet the targeted population are school students.

The Shamiri intervention is delivered by youth lay-providers aged only 18-to-22 (see Venturo-Conerly et al., 2021 for more information on the selection and training of youth lay-providers.) The idea is that a near peer model may be effective for low-touch psychotherapy like the Shamiri intervention. We have investigated this premise in 4 RCTs in Kenya already (many of which have been approved by KUERC). We describe the rationale for use youth lay-providers as group leaders on page 12 of the proposal under the section “Selection of Group Leaders”.

5. Protection of research participants lacks significant details and specific steps the researcher will take.

We are happy that we have put in place a robust participant management protocol for this study. We have highlighted potential risks on Page 16 and have also indicated our strategies to mitigate these risks. This is accompanied with a detailed an emergency protocol on page 14 which all members of the study team will be trained to follow. Nonetheless, we anticipate minimal risk to participants’ health and safety.

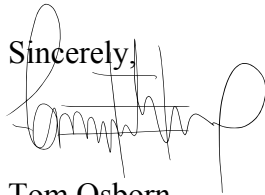
6. Will the research participants consent or assent? Some of them are below consenting age (12-17yrs).

We will recruit participants who may be below consenting age and as such, we request that KUERC allow us to work with the school administration to obtain consent from the parents of the minors. In addition to this, all minors will also provide assent prior to their participation in this study. This is highlighted in the informed consent/assent section on page 16.

7. Addition of qualitative indicators.

This study aims to provide insight into the unique experience of students and group leaders in each model of dissemination. For a more detailed and comprehensive outlook of these experiences, we wish to supplement our data with qualitative indicators through interviews of a small sub-sample (~20 participants). This addition is described in detail on page 19 of the proposal and is highlighted in yellow for easy reference. We have also added this to the consent/assent forms on pages 29 and 33.

I thank you again for your consideration of this proposal and hope this provides some clarity. We look forward to hearing back from you. Please let us know if you have any questions or comments.

Sincerely,


Tom Osborn
On behalf of the Shamiri study team